

SAMPLE Clinic Performance Review

Sample Medical Group (synthetic) · 12 providers, Jan 2025 - Aug 2026 · Part 1: provider productivity · JT Healthcare Analytics

89,041

Work RVUs (wRVUs), 20 months,
CMS 2026 values

96.7%

Median provider vs benchmark,
per clinical FTE

1

Provider below 80% of benchmark

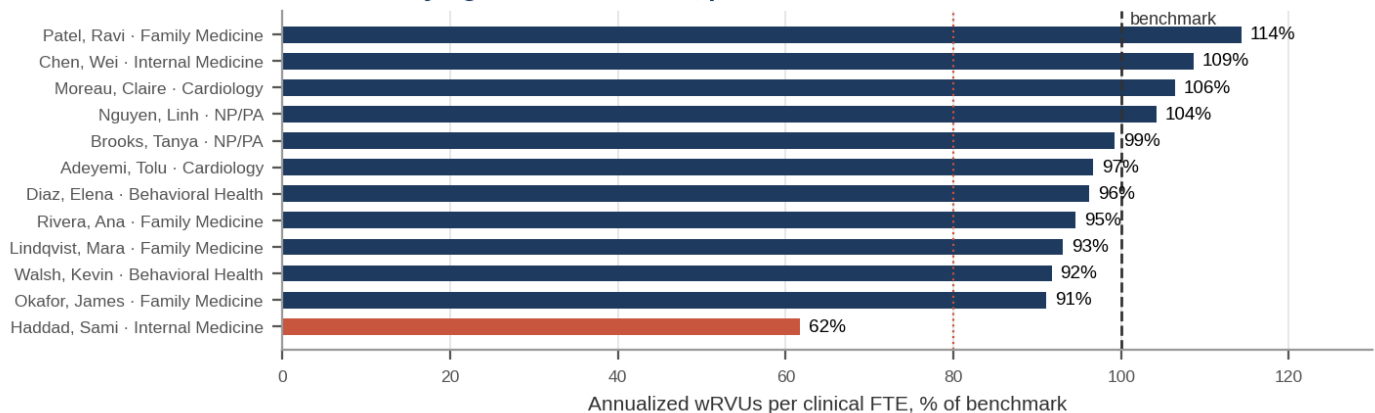
1,305

wRVUs available from consistent
G2211 use

What we found

- Median provider produces **96.7%** of benchmark per clinical FTE. **Dr. Sami Haddad** is at **61.8%**, which puts pay at **\$91 per wRVU** vs \$59 for the practice's other physicians. Schedule templates, panel size and no-shows explain most gaps like this; we check those before anyone talks about effort.
- Possible undercoding:** Dr. James Okafor bills level 4-5 on **21.8%** of visits vs **53.1%** for same-specialty peers. A 15-note documentation review settles whether visits are being under-billed.
- Audit risk:** Dr. Ravi Patel bills the top level (99215/99205) on **45.3%** of visits vs **18.0%** for peers. Payers notice patterns like this; documentation should be confirmed before they ask.
- G2211** (the add-on for ongoing primary care, 0.33 wRVU) is used on 4% and 7% of established visits by Dr. Mara Lindqvist and Linh Nguyen, PA, vs about 50% for colleagues: about **1,305 wRVUs** a year if documentation supports it.
- Benchmarks here are illustrative round numbers; a real review uses the practice's licensed survey (MGMA, AMGA) or contract targets.

Productivity against benchmark, per clinical FTE



Provider scorecard

Provider	Specialty	FTE	wRVUs / FTE / yr	% of bench.	Level 4-5	G2211	Pay / wRVU
Walsh, Kevin	Behavioral Health	0.8	2,569	92%	—	—	\$38
Diaz, Elena	Behavioral Health	1.0	2,694	96%	—	—	\$44
Adeyemi, Tolu	Cardiology	1.0	7,539	97%	62%	—	\$69
Moreau, Claire	Cardiology	0.9	8,300	106%	58%	—	\$68
Okafor, James	Family Medicine	1.0	4,373	91%	22%	44%	\$59
Lindqvist, Mara	Family Medicine	0.8	4,468	93%	59%	4%	\$60
Rivera, Ana	Family Medicine	1.0	4,541	95%	55%	52%	\$58
Patel, Ravi	Family Medicine	1.0	5,490	114%	82%	55%	\$49
Haddad, Sami	Internal Medicine	1.0	3,026	62%	57%	47%	\$91
Chen, Wei	Internal Medicine	1.0	5,326	109%	62%	55%	\$53
Brooks, Tanya	NP / PA primary care	1.0	3,771	99%	46%	41%	\$34
Nguyen, Linh	NP / PA primary care	0.9	3,960	104%	55%	7%	\$34

Part 2: quality gaps

Sample Medical Group (synthetic): 1,200 Medicare Advantage patients across 5 clinic sites · claims and enrollment, Jan 2025 - Aug 2026

4 of 10	5 of 10	2,020	61%
Measures that met target in 2025	On pace to meet target in 2026	Open gaps closable by Dec 31 (871 patients)	Colorectal screening heading for (target 72%)

Measures: last year, this year so far, and where the year is heading

Measure	2025 final	Same date 2025	2026 so far	Projected 2026	Target	Patients to close
Breast cancer screening (women 52-74)	44.5%	30.4%	55.2%	69.3%	74%	64
Colorectal cancer screening (46-75)	47.7%	36.6%	50.1%	61.2%	72%	159
Diabetes: HbA1c tested (18-75)	86.8%	58.9%	67.5%	95.4%	90%	65
Diabetes: HbA1c below 8%	61.1%	42.1%	48.6%	67.7%	65%	47
Diabetes: HbA1c above 9% or not tested *	29.7%	51.9%	43.4%	21.2%	≤ 20%	67
Diabetes: eye exam	59.1%	42.1%	40.9%	57.9%	68%	78
Diabetes: kidney health (eGFR + uACR)	46.3%	31.9%	29.7%	44.1%	45%	58
Blood pressure controlled <140/90	76.2%	71.8%	75.2%	79.7%	70%	–
Annual wellness visit (65+)	65.8%	50.7%	51.4%	66.4%	60%	84
Seen within 7 days after hospital discharge	60.9%	62.3%	59.4%	58.0%	50%	–

Highlighted: heading below target at the current pace. * Inverse measure: patients with no HbA1c result this year count as not controlled. Projection = 2026 so far + the gain 2025 made after the same date.

By clinic site: projected 2026 rate (red = below target)

Site	Mammo-gram	Colo-rectal	A1c test	A1c <8%	A1c >9%	Eye exam	Kidney	BP	AWV	7-day f/u
Lakeside	91%	69%	87%	68%	28%	63%	59%	90%	68%	83%
Northgate	46%	52%	85%	49%	40%	23%	32%	76%	63%	46%
Oak Hill	65%	60%	100%	75%	8%	54%	46%	68%	64%	66%
Peachtree	64%	62%	98%	66%	23%	69%	45%	87%	71%	54%
Riverside	83%	65%	94%	74%	17%	72%	41%	81%	67%	61%

What we found

- **2,020** open gaps sit with **871** patients; 286 of them have three or more, so one visit can close several. The outreach list ranks patients by open gaps and time since the last visit.
- Northgate closes the fewest gaps (48% of eligible measures met vs 61% at Lakeside): eye exams (23%), kidney tests (32%) and mammograms (46%) are the gap. Lakeside's standing orders and pre-visit planning are the model to copy.
- Colorectal screening only counts colonoscopies that appear in the claims sent; older ones need supplemental history before outreach. Same for eye exams done outside the network.

What the practice receives and what it costs

Service	Deliverable	Fixed fee
Provider productivity scorecard	wRVUs per provider and FTE vs your benchmarks, coding mix vs peers, G2211 and care-management use, pay per wRVU; 30-minute review	\$2,500-\$5,000; quarterly refresh \$750-\$1,500/quarter
Quality gaps review	Measures, projection and site comparison as above; outreach list with next step per patient	\$2,500-\$5,000; monthly gap list \$400-\$900/month
Clinic Performance Review	Both of the above plus the denial and underpayment review	\$6,000-\$12,000
Power BI dashboard (optional)	The same views as a Power BI file on your data, refreshed with each update	\$1,500-\$3,500 per dashboard

Needs a charge or productivity export (provider, date, CPT, modifier, units) and, for quality, claims with diagnoses plus a patient list with date of birth, sex and PCP site. Business Associate Agreement before any patient data. Measures are simplified claims-based versions of HEDIS/CMS Star logic for finding gaps, not certified HEDIS rates. wRVUs: CMS 2026 Physician Fee Schedule (RVU26D). Synthetic data; any resemblance to real people is coincidental.

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